

For the use only of a registered Medical Practitioner or a Hospital or a Laboratory

PACKAGE INSERT
LETTRA
Levetiracetam Injection USP
(Concentrate for Solution for Infusion) 100 mg/ml
5 mL Vial

Name and strengths of Active Ingredient

Each ml contains 100 mg of levetiracetam USP.

Each 5 ml vial contains 500 mg of levetiracetam USP.

Product Description

LETTRA Levetiracetam Injection USP (Concentrate for Solution for Infusion) 100 mg/ml is a clear, colourless solution packed as 5ml in 5ml USP Type-I clear glass tubular vial, with 20mm bromo butyl rubber closure and 20mm orange flip off seal. Each ml contains 100mg of Levetiracetam USP. Each 5 ml vial contains 500 mg of levetiracetam USP. Upon dilution, it appears as a clear colourless solution.

Pharmacodynamics

Mechanism of action

The mechanism of action of levetiracetam still remains to be fully elucidated. Levetiracetam does not alter basic cell characteristics and normal neurotransmission.

Levetiracetam affects intraneuronal Ca^{2+} levels by partial inhibition of N-type Ca^{2+} currents and by reducing the release of Ca^{2+} from intraneuronal stores. In addition, it partially reverses the reductions in GABA- and glycine-gated currents induced by zinc and β -carbolines. Furthermore, levetiracetam binds to a specific site in rodent brain tissue. This binding site is the synaptic vesicle protein 2A, believed to be involved in vesicle fusion and neurotransmitter exocytosis. Levetiracetam and related analogues show a rank order of affinity for binding to the synaptic vesicle protein 2A which correlates with the potency of their anti-seizure protection. The interaction between levetiracetam and the synaptic vesicle protein 2A seems to contribute to the antiepileptic mechanism of action of the medicinal product.

Pharmacodynamic effects

Levetiracetam induces seizure protection without having a pro-convulsant effect in animals and the primary metabolite is inactive.

Levetiracetam has a broad-spectrum pharmacological profile in man.

Pharmacokinetics

There is no safety concerns on the pharmacokinetic and safety profiles of levetiracetam.

Levetiracetam is a highly soluble and permeable compound. The pharmacokinetic profile is linear with low intra- and inter-subject variability. There is no modification of the clearance after repeated administration. The pharmacokinetic profile of levetiracetam is time independent.

There is no evidence for any relevant gender, race or circadian variability.

Distribution

No tissue distribution data are available in humans.

Neither levetiracetam nor its primary metabolite are significantly bound to plasma proteins.

The volume of distribution of levetiracetam is approximately 0.5 to 0.7 l/kg, a value close to the total body water volume.

Biotransformation

Levetiracetam is not extensively metabolised in humans. The major metabolic pathway is an enzymatic hydrolysis of the acetamide group. Production of the primary metabolite, ucb L057, is not supported by liver cytochrome P₄₅₀ isoforms. Hydrolysis of the acetamide group was measurable in a large number of tissues including blood cells. The metabolite ucb L057 is pharmacologically inactive.

Two minor metabolites is identified. One was obtained by hydroxylation of the pyrrolidone ring and the other one by opening of the pyrrolidone ring.

There is no enantiomeric interconversion for either levetiracetam or its primary metabolite.

The interaction of Levetiracetam with other substances, or *vice versa*, is unlikely.

Elimination

The plasma half-life in adults is 7 ± 1 hours and do not vary either with dose, route of administration or repeated administration. The mean total body clearance is 0.96 ml/min/kg.

The major route of excretion was via urine, accounting for a mean 95 % of the dose (approximately 93 % of the dose was excreted within 48 hours). Excretion via faeces accounted for only 0.3 % of the dose.

Levetiracetam is excreted by glomerular filtration with subsequent tubular reabsorption and the primary metabolite is also excreted by active tubular secretion in addition to glomerular filtration. Levetiracetam elimination is correlated to creatinine clearance.

Elderly

In elderly, the half-life is increased. This is related to the decrease in renal function in this population (see Recommended Dosage section).

Renal impairment

The apparent body clearance of both levetiracetam and of its primary metabolite is correlated to the creatinine clearance. It is therefore recommended to adjust the maintenance daily dose of Levetiracetam, based on creatinine clearance in patients with moderate and severe renal impairment (see Recommended Dosage section).

Hepatic impairment

There is no relevant modification of the clearance of levetiracetam in mild and moderate hepatic impairment.

Paediatric population

Children (4 to 12 years)

The pharmacokinetics in paediatric patients has not been investigated after intravenous administration. However, based on the pharmacokinetic characteristics of levetiracetam, the pharmacokinetics in adults after intravenous administration and the pharmacokinetics in children after oral administration, the exposure (AUC) of levetiracetam is expected to be similar in paediatric patients aged 4 to 12 years after intravenous and oral administration.

Indication

Levetiracetam is indicated as monotherapy in the treatment of partial onset seizures with or without secondary generalisation in adults and adolescents from 16 years of age with newly diagnosed epilepsy.

Levetiracetam is indicated as adjunctive therapy

- in the treatment of partial onset seizures with or without secondary generalisation in adults, adolescents and children from 4 years of age with epilepsy.
- in the treatment of myoclonic seizures in adults and adolescents from 12 years of age with Juvenile Myoclonic Epilepsy.
- in the treatment of primary generalised tonic-clonic seizures in adults and adolescents from 12 years of age with Idiopathic Generalised Epilepsy.

Levetiracetam concentrate is an alternative for patients (adults and children from 4 years of age) when oral administration is temporarily not feasible.

Recommended Dosage

Posology

Levetiracetam therapy can be initiated with either intravenous or oral administration.

Conversion to or from oral to intravenous administration can be done directly without titration. The total daily dose and frequency of administration should be maintained.

Monotherapy for adults and adolescents from 16 years of age

The recommended starting dose is 250 mg twice daily which should be increased to an initial therapeutic dose of 500 mg twice daily after two weeks. The dose can be further increased by 250 mg twice daily every two weeks depending upon the clinical response. The maximum dose is 1500 mg twice daily.

Add-on therapy for adults (≥18 years) and adolescents (12 to 17 years) weighing 50 kg or more

The initial therapeutic dose is 500 mg twice daily. This dose can be started on the first day of treatment.

Depending upon the clinical response and tolerability, the daily dose can be increased up to 1,500 mg twice daily.

Dose changes can be made in 500 mg twice daily increases or decreases every two to four weeks.

Duration of treatment

There is no experience with administration of intravenous levetiracetam for longer period than 4 days.

Special populations

Elderly (65 years and older)

Adjustment of the dose is recommended in elderly patients with compromised renal function (see “Renal impairment” below).

Renal impairment

The daily dose must be individualised according to renal function.

For adult patients, refer to the following table and adjust the dose as indicated. To use this dosing table, an estimate of the patient's creatinine clearance (CLcr) in ml/min is needed. The CLcr in ml/min may be estimated from serum creatinine (mg/dl) determination, for adults and adolescents weighing 50 kg or more, the following formula:

$$\text{CLcr (ml/min)} = \frac{[140 - \text{age (year)}] \times \text{weight (kg)}}{72 \times \text{serum creatinine (mg/dl)}} \quad (\times 0.85 \text{ for women})$$

Then CLcr is adjusted for body surface area (BSA) as follows:

$$\text{CLcr (ml/min/1.73m}^2\text{)} = \frac{\text{CLcr (ml/min)}}{\text{BSA subject (m}^2\text{)}} \times 1.73$$

Dosing adjustment for adult and adolescents patients weighing more than 50 kg with impaired renal function:

Group	Creatinine clearance (ml/min/1.73m ²)	Dose and frequency
Normal	≥ 80	500 to 1,500 mg twice daily
Mild	50-79	500 to 1,000 mg twice daily
Moderate	30-49	250 to 750 mg twice daily
Severe	< 30	250 to 500 mg twice daily
End-stage renal disease patients undergoing dialysis ⁽¹⁾	-	500 to 1,000 mg once daily ⁽²⁾

⁽¹⁾ A 750 mg loading dose is recommended on the first day of treatment with levetiracetam.

⁽²⁾ Following dialysis, a 250 to 500 mg supplemental dose is recommended.

For children with renal impairment, levetiracetam dose needs to be adjusted based on the renal function as levetiracetam clearance is related to renal function. This recommendation is based on a study in adult renally impaired patients.

The CLcr in ml/min/1.73 m² may be estimated from serum creatinine (mg/dl) determination, for young adolescents and children using the following formula (Schwartz formula):

$$\text{CLcr (ml/min/1.73m}^2\text{)} = \frac{\text{Height (cm)} \times \text{ks}}{\text{Serum Creatinine (mg/dl)}}$$

ks= 0.55 in Children to less than 13 years and in adolescent female; ks= 0.7 in adolescent male

Dosing adjustment for children and adolescents patients weighing less than 50 kg with impaired renal function:

Group	Creatinine clearance (ml/min/1.73m ²)	Dose and frequency
		Children from 4 years and adolescents weighing less than 50 kg
Normal	≥ 80	10 to 30 mg/kg (0.10 to 0.30 ml/kg) twice daily
Mild	50-79	10 to 20 mg/kg (0.10 to 0.20 ml/kg) twice daily
Moderate	30-49	5 to 15 mg/kg (0.05 to 0.15 ml/kg) twice daily
Severe	< 30	5 to 10 mg/kg (0.05 to 0.10 ml/kg) twice daily
End-stage renal disease patients undergoing dialysis	--	10 to 20 mg/kg (0.10 to 0.20 ml/kg) once daily ^{(1) (2)}

⁽¹⁾ A 15 mg/kg (0.15 ml/kg) loading dose is recommended on the first day of treatment with levetiracetam.

⁽²⁾ Following dialysis, a 5 to 10 mg/kg (0.05 to 0.10 ml/kg) supplemental dose is recommended.

Hepatic impairment

No dose adjustment is needed in patients with mild to moderate hepatic impairment. In patients with severe hepatic impairment, the creatinine clearance may underestimate the renal insufficiency. Therefore a 50 % reduction of the daily maintenance dose is recommended when the creatinine clearance is < 60 ml/min/1.73 m².

Paediatric population

The physician should prescribe the most appropriate pharmaceutical form, presentation and strength according to age, weight and dose.

Monotherapy

The safety and efficacy of Levetiracetam in children and adolescents below 16 years as monotherapy treatment have not been established.

No data are available.

Add-on therapy for children aged 4 to 11 years and adolescents (12 to 17 years) weighing less than 50 kg

Levetiracetam oral solution is the preferred formulation for use in children under the age of 6 years.

The initial therapeutic dose is 10 mg/kg twice daily.

Depending upon the clinical response and tolerability, the dose can be increased up to 30 mg/kg twice daily. Dose changes should not exceed increases or decreases of 10 mg/kg twice daily every two weeks. The lowest effective dose should be used.

Dose in children 50 kg or greater is the same as in adults.

Dose recommendations for children and adolescents:

Weight	Starting dose:	Maximum dose:
	10 mg/kg twice daily	30 mg/kg twice daily
15 kg ⁽¹⁾	150 mg twice daily	450 mg twice daily
20 kg ⁽¹⁾	200 mg twice daily	600 mg twice daily
25 kg	250 mg twice daily	750 mg twice daily
From 50 kg ⁽²⁾	500 mg twice daily	1500 mg twice daily

⁽¹⁾ Children 25 kg or less should preferably start the treatment with Levetiracetam 100 mg/ml oral solution.

⁽²⁾ Dose in children and adolescents 50 kg or more is the same as in adults.

Add-on therapy for infants and children less than 4 years

The safety and efficacy of Levetiracetam concentrate for solution for infusion in infants and children less than 4 years have not been established.

Route of Administration

For intravenous use only

Instruction for use

Levetiracetam concentrate is for intravenous use only and the recommended dose must be diluted in at least 100 ml of a compatible diluent and administered intravenously as a 15-minute intravenous infusion.

See Table 1 for the recommended preparation and administration of Levetiracetam concentrate for solution for infusion to achieve a total daily dose of 500 mg, 1,000 mg, 2,000 mg, or 3,000 mg in two divided doses.

Table 1. Preparation and administration of Levetiracetam concentrate for solution for infusion

Dose	Withdrawal Volume	Volume of Diluent	Infusion Time	Frequency of Administration	Total Daily Dose
250 mg	2.5 ml (half 5 ml vial)	100 ml	15 minutes	Twice daily	500 mg/day
500 mg	5 ml (one 5 ml vial)	100 ml	15 minutes	Twice daily	1000 mg/day
1000 mg	10 ml (two 5 ml vials)	100 ml	15 minutes	Twice daily	2000 mg/day
1500 mg	15 ml (three 5 ml vials)	100 ml	15 minutes	Twice daily	3000 mg/day

This medicinal product is for single use only, any unused solution should be discarded.

From a microbiological point of view, the product should be used immediately after dilution. If not used immediately, in-use storage time and conditions prior to use are the responsibility of the user and would normally not be longer than 24 hours when stored below 30°C for Sodium chloride 9 mg/ml (0.9%) solution for injection.

Diluents:

- Sodium chloride 9 mg/ml (0.9%) solution for injection

Medicinal product with particulate matter or discoloration should not be used.

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

Contraindications

Hypersensitivity to the active substance or other pyrrolidone derivatives or to any of the excipients listed in this insert.

Warnings and Precautions

Discontinuation

If levetiracetam has to be discontinued it is recommended to withdraw it gradually (*e.g.* in adults and adolescents weighing more than 50 kg: 500 mg decreases twice daily every two to four weeks; in children and adolescents weighing less than 50 kg: dose decrease should not exceed 10 mg/kg twice daily every two weeks).

Renal impairment

The administration of levetiracetam to patients with renal impairment may require dose adjustment. In patients with severely impaired hepatic function, assessment of renal function is recommended before dose selection (see Recommended Dosage section).

Acute Kidney injury

The use of levetiracetam has been very rarely associated with acute kidney injury, with a time to onset ranging from a few days to several months.

Blood cell counts

Rare cases of decreased blood cell counts (neutropenia, agranulocytosis, leucopenia, thrombocytopenia and pancytopenia) have been described in association with levetiracetam administration, generally at the beginning of the treatment. Complete blood cell counts are advised in patients experiencing important weakness, pyrexia, recurrent infections or coagulation disorders (Undesirable Effects section).

Suicide

Suicide, suicide attempt, suicidal ideation and behaviour have been reported in patients treated with anti-epileptic agents (including levetiracetam). A meta-analysis of randomized placebo-controlled trials of anti-epileptic medicinal products has shown a small increased risk of suicidal thoughts and behaviour. The mechanism of this risk is not known.

Therefore patients should be monitored for signs of depression and/or suicidal ideation and behaviours and appropriate treatment should be considered. Patients (and caregivers of patients) should be advised to seek medical advice should signs of depression and/or suicidal ideation or behaviour emerge.

Abnormal and aggressive behaviours

Levetiracetam may cause psychotic symptoms and behavioural abnormalities including irritability and aggressiveness. Patients treated with levetiracetam should be monitored for developing psychiatric signs suggesting important mood and/or personality changes. If such behaviours are noticed, treatment adaptation or gradual discontinuation should be considered.

Worsening of seizures

As with other types of antiepileptic drugs, levetiracetam may rarely exacerbate seizure frequency or severity. This paradoxical effect was mostly reported within the first month after levetiracetam initiation or increase of the dose, and was reversible upon drug discontinuation or dose decrease. Patients should be advised to consult their physician immediately in case of aggravation of epilepsy.

Electrocardiogram QT interval prolongation

Rare cases of ECG QT interval prolongation have been observed during the post-marketing surveillance. Levetiracetam should be used with caution in patients with QTc-interval prolongation, in patients concomitantly treated with drugs affecting the QTc-interval, or in patients with relevant pre-existing cardiac disease or electrolyte disturbances.

Paediatric population

Available data in children did not suggest impact on growth and puberty. However, long term effects on learning, intelligence, growth, endocrine function, puberty and childbearing potential in children remain unknown.

Interactions with Other Medicaments

Antiepileptic medicinal products

Pre-marketing data from clinical studies conducted in adults indicate that levetiracetam did not influence the serum concentrations of existing antiepileptic medicinal products (phenytoin, carbamazepine, valproic acid, phenobarbital, lamotrigine, gabapentin and primidone) and that these antiepileptic medicinal products did not influence the pharmacokinetics of levetiracetam.

As in adults, there is no evidence of clinically significant medicinal product interactions in paediatric patients receiving up to 60 mg/kg/day levetiracetam.

A retrospective assessment of pharmacokinetic interactions in children and adolescents with epilepsy (4 to 17 years) confirmed that adjunctive therapy with orally administered levetiracetam did not influence the steady-state serum concentrations of concomitantly administered carbamazepine and valproate. However, data suggested a 20 % higher levetiracetam clearance in children taking enzyme-inducing antiepileptic medicinal products. Dose adjustment is not required.

Probenecid

Probenecid (500 mg four times daily), a renal tubular secretion blocking agent, has been shown to inhibit the renal clearance of the primary metabolite, but not of levetiracetam. Nevertheless, the concentration of this metabolite remains low.

Methotrexate

Concomitant administration of levetiracetam and methotrexate has been reported to decrease methotrexate clearance, resulting in increased/prolonged blood methotrexate concentration to potentially toxic levels. Blood methotrexate and levetiracetam levels should be carefully monitored in patients treated concomitantly with the two drugs

Oral contraceptives and other pharmacokinetics interactions

Levetiracetam 1,000 mg daily did not influence the pharmacokinetics of oral contraceptives (ethinyl-estradiol and levonorgestrel); endocrine parameters (luteinizing hormone and progesterone) were not modified. Levetiracetam 2,000 mg daily did not influence the pharmacokinetics of digoxin and warfarin; prothrombin times were not modified. Co-administration with digoxin, oral contraceptives and warfarin did not influence the pharmacokinetics of levetiracetam.

Alcohol

No data on the interaction of levetiracetam with alcohol are available.

Fertility, pregnancy and lactation

Women of child bearing potential

Specialist advice should be given to women who are of childbearing potential. Treatment with levetiracetam should be reviewed when a woman is planning to become pregnant. As with all antiepileptic medicines, sudden discontinuation of levetiracetam should be avoided as this may lead to breakthrough seizures that could have serious consequences for the woman and the unborn child. Monotherapy should be preferred whenever possible because therapy with multiple antiepileptic medicines AEDs could be associated with a higher risk of congenital malformations than monotherapy, depending on the associated antiepileptics.

Pregnancy

A large amount of postmarketing data on pregnant women exposed to levetiracetam monotherapy (more than 1800, among which in more than 1500 exposure occurred during the 1st trimester) do not suggest an increase in the risk for major congenital malformations. Only limited evidence is available on the neurodevelopment of children exposed to Levetiracetam monotherapy in utero. However, current epidemiological studies (on about 100 children) do not suggest an increased risk of neurodevelopmental disorders or delays.

Levetiracetam can be used during pregnancy, if after careful assessment it is considered clinically needed. In such case, the lowest effective dose is recommended.

Physiological changes during pregnancy may affect levetiracetam concentration. Decrease in levetiracetam plasma concentrations has been observed during pregnancy. This decrease is more pronounced during the third trimester (up to 60% of baseline concentration before pregnancy). Appropriate clinical management of pregnant women treated with levetiracetam should be ensured.

Breastfeeding

Levetiracetam is excreted in human breast milk. Therefore, breast-feeding is not recommended.

However, if levetiracetam treatment is needed during breastfeeding, the benefit/risk of the treatment should be weighed considering the importance of breastfeeding.

Fertility

No impact on fertility was detected in animal studies No clinical data are available, potential risk for human is unknown.

Adverse Effects/ Undesirable Effects

Summary of the safety profile

The most frequently reported adverse reactions were nasopharyngitis, somnolence, headache, fatigue and dizziness. The safety profile of levetiracetam is generally similar across age groups (adult and paediatric patients) and across the approved epilepsy indications.

Tabulated list of adverse reactions

Adverse reactions are listed in the following table per System Organ Class and per frequency. Adverse reactions are presented in the order of decreasing seriousness and their frequency as follows: very common; common; uncommon; rare and very rare.

MedDRA SOC	Frequency category			
	Very common	Common	Uncommon	Rare
Infections and infestations	Nasopharyngitis			Infection
Blood and lymphatic system disorders			Thrombocytopenia, leukopenia	Pancytopenia, neutropenia, agranulocytosis
Immune system disorders				Drug reaction with eosinophilia and systemic symptoms (DRESS), Hypersensitivity (including angioedema and anaphylaxis)
Metabolism and nutrition disorders		Anorexia	Weight decreased, weight increase	Hyponatraemia
Psychiatric disorders		Depression, hostility / aggression, anxiety, insomnia, nervousness/irritability	Suicide attempt, suicidal ideation, psychotic disorder, abnormal behaviour, hallucination, anger, confusional state, panic attack, affect lability/mood swings, agitation	Completed suicide, personality disorder, thinking abnormal, delirium
Nervous system disorders	Somnolence, headache	Convulsion, balance disorder, dizziness, lethargy, tremor	Amnesia, memory impairment, coordination abnormal/ataxia, paraesthesia, disturbance in attention	Choreoathetosis, dyskinesia, hyperkinesia, gait disturbance, encephalopathy
Eye disorders			Diplopia, vision blurred	
Ear and labyrinth disorders		Vertigo		
Respiratory, thoracic and mediastinal disorders		Cough		
Gastrointestinal disorders		Abdominal pain, diarrhoea, dyspepsia, vomiting, nausea		Pancreatitis
Hepatobiliary disorders			Liver function test abnormal	Hepatic failure, hepatitis
Renal and Urinary Disorders				Acute kidney injury
Skin and subcutaneous tissue disorders		Rash	Alopecia, eczema, pruritus,	Toxic epidermal necrolysis, Stevens-Johnson syndrome, erythema multiforme

Musculoskeletal and connective tissue disorders			Muscular weakness, myalgia	Rhabdomyolysis and blood creatine phosphokinase increased*
General disorders and administration site conditions		Asthenia/fatigue		
Injury, poisoning and procedural complications			Injury	

* Prevalence is significantly higher in Japanese patients when compared to non-Japanese patients.

Description of selected adverse reactions

The risk of anorexia is higher when levetiracetam is co administered with topiramate. In several cases of alopecia, recovery was observed when levetiracetam was discontinued. Bone marrow suppression was identified in some of the cases of pancytopenia. Cases of encephalopathy have been rarely observed after levetiracetam administration. These undesirable effects generally occurred at the beginning of the treatment (few days to a few months) and were reversible after treatment discontinuation.

Paediatric population

The adverse reaction profile of levetiracetam is generally similar across age groups and across the approved epilepsy indications.

Effects on ability to drive and use machines

Levetiracetam has minor or moderate influence on the ability to drive and use machines. Due to possible different individual sensitivity, some patients might experience somnolence or other central nervous system related symptoms, especially at the beginning of treatment or following a dose increase. Therefore, caution is recommended in those patients when performing skilled tasks, *e.g.* driving vehicles or operating machinery. Patients are advised not to drive or use machines until it is established that their ability to perform such activities is not affected.

Overdose and Treatment

Symptoms

Somnolence, agitation, aggression, depressed level of consciousness, respiratory depression and coma were observed with Levetiracetam overdoses.

Management of overdose

There is no specific antidote for levetiracetam. Treatment of an overdose will be symptomatic and may include haemodialysis. The dialyser extraction efficiency is 60 % for levetiracetam and 74 % for the primary metabolite.

List of Excipients

Sodium Acetate Trihydrate USP
Sodium Chloride USP
Glacial Acetic Acid USP
Water for Injection USP

Incompatibilities

This medicinal product must not be mixed with other medicinal products except those mentioned in Instruction for Use.

Shelf-life

Shelf life in the medicinal product as packaged for sale: 36 months

Shelf life after dilution according to directions: From a microbiological point of view, the product should be used immediately after dilution. If not used immediately, in-use storage time and conditions prior to use are the responsibility of the user and would normally not be longer than 24 hours when stored below 30°C for Sodium chloride 9 mg/ml (0.9%) solution for injection.

Storage Conditions

Unopened: Store below 30 °C

After dilution according to direction: From a microbiological point of view, the product should be used immediately after dilution. If not used immediately, in-use storage time and conditions prior to use are the responsibility of the user and would normally not be longer than 24 hours when stored below 30°C for Sodium chloride 9 mg/ml (0.9%) solution for injection.

Dosage forms and packaging available

LETTRA Levetiracetam Injection USP (Concentrate for Solution for Infusion) 100 mg/ml is packed as 5ml in 5ml USP Type-I clear glass tubular vial, with 20mm bromo butyl rubber closure and 20mm orange flip off seal. 10 vials or 25 vials are packed in printed carton with printed insert.

Manufacturer

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